Discharge Summary for Robert King, MRN 010-77345, born 1969-10-28.

Diagnoses at discharge included acute kidney injury of pre-renal origin and dehydration.

During the hospital stay, Mr. King's condition showed significant improvement following the administration of intravenous fluids.

Medications prescribed during the hospitalization included normal saline IV fluids and ondansetron 4 mg orally disintegrating tablets every 8 hours as needed for nausea.

Key observations noted during the hospital course included a creatinine level of 2.1 mg/dL upon admission, which decreased to 1.4 mg/dL at discharge, and a BUN level of 38 mg/dL.

Inpatient procedures included IV fluid resuscitation.

The plan for Mr. King's post-discharge care included oral hydration, avoidance of nonsteroidal anti-inflammatory drugs, and a repeat basic metabolic panel with his primary care physician in 3 days.

DISCHARGE SUMMARY

Patient Robert King, MRN 010-77345, born 1969-10-28, was admitted on 2026-05-10 and discharged on 2026-05-11.

DIAGNOSES
- Acute kidney injury of pre-renal origin
- Dehydration

HOSPITAL COURSE
The patient's condition showed significant improvement following the administration of intravenous fluids.

MEDICATIONS
- Normal saline was administered intravenously during the hospital stay
- Ondansetron 4 mg orally disintegrating tablets were given every 8 hours as needed for nausea

OBSERVATIONS
- The patient's creatinine level was 2.1 mg/dL upon admission and decreased to 1.4 mg/dL at discharge
- Blood urea nitrogen (BUN) was 38 mg/dL

PROCEDURES
- The patient underwent intravenous fluid resuscitation

PLAN
The patient will be advised to maintain oral hydration, avoid nonsteroidal anti-inflammatory drugs (NSAIDs), and have a repeat basic metabolic panel (BMP) with their primary care physician (PCP) in three days.

DISCHARGE SUMMARY

Patient Robert King, MRN 010-77345, born 1969-10-28, was admitted on 2026-05-10 and discharged on 2026-05-11.

DIAGNOSES
- The patient was diagnosed with acute kidney injury of pre-renal origin.
- Dehydration was also identified as a contributing factor.

HOSPITAL COURSE

The patient's condition showed significant improvement following the administration of intravenous fluids.

MEDICATIONS
- Normal saline IV fluids were administered during the hospital stay.
- Ondansetron 4 mg ODT was prescribed every 8 hours as needed for nausea.

OBSERVATIONS
- Upon admission, the patient's creatinine level was 2.1 mg/dL, which decreased to 1.4 mg/dL at discharge.
- The patient's BUN level was 38 mg/dL.

PROCEDURES
- The patient underwent IV fluid resuscitation to address dehydration.

PLAN
The patient will be advised to maintain oral hydration and avoid the use of nonsteroidal anti-inflammatory drugs (NSAIDs). A repeat blood panel (BMP) is scheduled with the patient's primary care physician (PCP) in 3 days.

DISCHARGE SUMMARY

Patient Robert King, MRN 010-77345, born 1969-10-28, was admitted on 2026-05-10 and discharged on 2026-05-11.

DIAGNOSES
- Acute kidney injury, classified as pre-renal in origin
- Dehydration

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PROCEDURES
- Intravenous fluid resuscitation was performed

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